DISCHARGE SUMMARY

Note type: Discharge Summary
Status: **Signed**

Patient: Lindqvist, Theodore A	**Sex:** M	**Admit date:** 10/19/2026 21:06
DOB: 03/14/1955 (71 y)	**Unit/Bed:** 4 West / 412-B	**Discharge date:** 10/24/2026 14:55
Attending: Priya Raman, MD (Hospitalist)	**LOS:** 5 days	**Admit source:** Emergency Dept
PCP: Maya Desai, MD, Orchard Family Medicine	**Code status:** Full Code	**Disposition:** Home w/ home O2 (self-care)
Consultants: Pulmonology (S. Brennan, MD); Tobacco Treatment Svc	**Language:** English	**Insurance:** Medicare A/B

ALLERGIES / INTOLERANCES

Penicillin (hives, 1980s). **Sulfamethoxazole-trimethoprim** (diffuse maculopapular rash). No known food or latex allergies.

DIAGNOSES

Principal (discharge) dx: Acute exacerbation of COPD (J44.1), GOLD 3 / group E, likely viral-triggered, complicated by acute on chronic hypoxemic respiratory failure (J96.21).
Secondary / active: (1) Paroxysmal atrial fibrillation on anticoagulation, RVR on admission, rate controlled. (2) Type 2 DM, steroid-induced hyperglycemia. (3) Tobacco use disorder, 50 pack-yr, current 1 ppd. (4) HTN. (5) HLD. (6) BPH.
Admission dx: Dyspnea; r/o pneumonia; r/o ACS.

PROCEDURES

None. No intubation or NIPPV required.

BRIEF HPI / REASON FOR ADMISSION

71M w/ severe COPD (FEV1 41% pred, 03/2025), pAF on apixaban, T2DM, active smoker, presented to ED w/ 4 days of progressive DOE, wheeze and increased purulent sputum after URI sx in household (grandchild). Using albuterol q2h at home w/o relief. In ED: RR 28, HR 134 irreg (AF w/ RVR), SpO2 84% RA, T 37.9 C. VBG 7.33/58. Diffuse exp wheeze, prolonged exp phase, accessory muscle use. CXR hyperinflation, no focal consolidation. Trop neg x2. Resp viral panel **positive for rhinovirus/enterovirus**.

HOSPITAL COURSE (BY PROBLEM)

#AECOPD w/ acute on chronic hypoxemic resp failure. Received DuoNeb q4h + albuterol q2h PRN, methylprednisolone 40 mg IV x1 then prednisone 40 mg PO daily (day 1 = 10/20; 5-day course per GOLD). Started doxycycline 100 mg PO BID for increased sputum purulence (PCN allergy). Required up to 4 L NC on HD1-2, weaned to RA at rest by HD4. Ambulatory oximetry on 10/24: SpO2 RA rest 91%, **nadir 85% w/ ambulation**, 92% on 2 L NC w/ ambulation; qualifies for home O2 with exertion and sleep. Pulm (Dr. Brennan) consulted: escalate maintenance from LAMA monotherapy (tiotropium HandiHaler) to LAMA/LABA (tiotropium-olodaterol Respimat); consider ICS add-on at f/u if eos >= 300 (eos 210 on admission, on steroids). Inhaler teach-back completed w/ RT x2; pt technique w/ HandiHaler was poor, Respimat technique adequate. Referral placed to pulmonary rehab.
#pAF w/ RVR. Likely precipitated by hypoxemia + frequent beta-agonist. Diltiazem 10 mg IV x2 in ED, then home diltiazem ER resumed; HR 70s-90s since HD2. CHA2DS2-VASc 3 (age, DM, HTN hx). Apixaban continued; dose remains 5 mg BID (age < 80, wt 74 kg, Cr 1.0). TSH 1.8.
#T2DM / steroid-induced hyperglycemia. A1c 7.4%. Metformin held on admission (contrast not given; held for acute illness), managed w/ SSI; BG peaks 250-310 in afternoons on prednisone. Metformin resumed 10/23. No basal insulin needed at d/c given steroid course ends 10/24 (see meds). Pt to check fingerstick BG BID x 1 wk.
#Tobacco use. Counseled by Tobacco Treatment Svc; pt motivated, quit date set for day of d/c. Started NRT patch 21 mg + gum 2 mg PRN. Warned re: **no smoking or open flame w/ home O2** (fire risk).
#BPH / sleep. Pt reports taking OTC diphenhydramine 50 mg qHS for sleep; had urinary retention on HD2 (bladder scan 640 mL, straight cath x1). Diphenhydramine **discontinued** (Beers list, anticholinergic). Voiding spontaneously since HD3, PVR 80 mL.
#HLD. Continue atorvastatin.
#Health maintenance. Influenza vaccine (2026-27) given 10/23/2026. Due for PCV20 and RSV vaccine; defer to PCP.

PERTINENT RESULTS

Lab	10/19 (admit)	10/22	10/24 (d/c)	Ref range
WBC (K/uL)	13.8 H	15.1 H*	12.2 H*	4.0-11.0
Hgb (g/dL)	15.9	15.2	15.0	13.5-17.5
Plt (K/uL)	248	262	271	150-400

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Lab	10/19 (admit)	10/22	10/24 (d/c)	Ref range
Na (mmol/L)	136	138	139	135-145
K (mmol/L)	3.4 L	4.1	4.3	3.5-5.1
Creatinine (mg/dL)	1.12	0.98	1.01	0.70-1.30
Glucose (mg/dL)	198 H	287 H	164 H	70-99
Mg (mg/dL)	1.7	2.0	--	1.7-2.4
BNP (pg/mL)	112 H	--	--	< 100
hs-Troponin I (ng/L)	9 / 11	--	--	< 20
Procalcitonin (ng/mL)	0.11	--	--	< 0.25
VBG pH / pCO2	7.33 / 58	7.38 / 49	--	7.32-7.43 / 41-51

* Leukocytosis attributed to corticosteroids (demargination); afebrile since HD2.

CXR PA/lat (10/19): Hyperinflated lungs w/ flattened hemidiaphragms. No focal consolidation, effusion or PTX. Stable 6 mm RUL nodule, unchanged from CT 04/2025.
ECG (10/19): AF w/ RVR, rate 132, no acute ST changes. **ECG (10/21):** AF, rate 84, QTc 448 ms.
Resp viral panel (10/19): Rhinovirus/enterovirus DETECTED. SARS-CoV-2, influenza A/B, RSV not detected.
Blood cx x2 (10/19): No growth at 5 days (final).

RESULTS PENDING AT DISCHARGE

Sputum culture (10/20): Prelim: moderate GNRs, ID and susceptibilities pending. **Responsible for f/u: Dr. Desai (PCP)**; result will route to PCP in-basket. If Pseudomonas or a doxycycline-resistant organism grows, PCP to contact pt re: change in antibiotic.

CONDITION AT DISCHARGE

Stable, improved. A&O x3. Ambulating independently ~150 ft w/ 2 L NC w/o distress. Speaking full sentences. Mild residual end-exp wheeze bilat. **D/C vitals:** T 36.8 C, HR 82 irreg, BP 128/74, RR 18, SpO2 92% RA at rest, Wt 74.1 kg. Decision-making capacity intact.

DISCHARGE MEDICATION RECONCILIATION

Reconciled against pt's home med list and pharmacy fill hx (CVS #4418) on 10/24/2026 by P. Raman, MD and J. Okoye, PharmD.

Medication	Dose / route / frequency	Status	Notes / last dose given
predniSONE 20 mg tab	40 mg (2 tabs) PO daily w/ breakfast	NEW, **COMPLETED**	5-day course 10/20-10/24. Final dose given 10/24 08:00. **No further doses; no taper needed.**
doxycycline hyclate 100 mg cap	100 mg PO BID x 2 more days	NEW	Last dose 10/26 PM (7-day course started 10/20). Take w/ full glass of water, stay upright 30 min. Separate from Ca, Mg, Fe, antacids, dairy by 2 h. Photosensitivity.
tiotropium-olodaterol (Stiolto Respimat) 2.5-2.5 mcg/actuation	2 inhalations INH once daily	NEW (replaces tiotropium HandiHaler)	Same time each day. Do not use for rescue. First dose given 10/23 09:00.
tiotropium (Spiriva HandiHaler) 18 mcg cap	--	**STOP**	Replaced by Stiolto above. Do NOT use both (duplicate LAMA).
albuterol HFA 90 mcg/actuation	2 puffs INH q4-6h PRN SOB/wheeze	CONTINUE	Use w/ spacer. Call if needing > q4h.
nicotine patch 21 mg/24 h	1 patch TD daily	NEW	Wks 1-6: 21 mg; wks 7-8: 14 mg; wks 9-10: 7 mg. Remove at bedtime if vivid dreams. Rotate sites.
nicotine polacrilex gum 2 mg	1 piece PO q1-2h PRN cravings, max 24/day	NEW	Chew-and-park technique.
apixaban 5 mg tab	5 mg PO BID	CONTINUE	Last dose 10/24 09:00. Next dose tonight.
diltiazem ER 180 mg cap (24 h)	180 mg PO daily	CONTINUE	Do not crush or chew.
metFORMIN 1000 mg tab	1000 mg PO BID w/ meals	CONTINUE (resumed 10/23)	Held 10/19-10/22.
atorvastatin 40 mg tab	40 mg PO qHS	CONTINUE	
tamsulosin 0.4 mg cap	0.4 mg PO daily 30 min after same meal	CONTINUE	

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Medication	Dose / route / frequency	Status	Notes / last dose given
diphenhydrAMINE 25 mg tab (OTC)	was taking 50 mg (2 tabs) PO qHS PRN sleep	**STOP**	Caused urinary retention. Avoid OTC "PM" sleep aids.
Oxygen (home, via NC)	2 L/min w/ exertion and during sleep	NEW	Target SpO2 88-92%. Vendor: BreatheWell Home Medical (555) 014-7780, delivered to home 10/24.

DISCHARGE INSTRUCTIONS

Diet: Carbohydrate-consistent (diabetic) diet. **Activity:** As tolerated; walk daily w/ O2 on, increase gradually. Use pursed-lip breathing w/ exertion. No driving while dyspneic.
Monitoring: Check fingerstick BG before breakfast and before dinner x 7 days and bring log to PCP. Check SpO2 w/ home pulse oximeter if short of breath.
O2 safety: No smoking, candles, gas stove or open flame within 10 ft of O2. Keep tubing away from heat.
Education provided (teach-back confirmed): COPD action plan (green/yellow/red zones, copy given), Respimat inhaler technique, home O2 use and safety, NRT use, AF/anticoagulation bleeding precautions, hypoglycemia/hyperglycemia sx. Pt and daughter (Karin, HCP) verbalized understanding.

FOLLOW-UP APPOINTMENTS

Follow-up with	When	Purpose
Maya Desai, MD (PCP), Orchard Family Medicine, (555) 014-3350	Thu 10/29/2026 10:15 AM	Post-hosp f/u, BG log review, sputum cx result, PCV20/RSV vaccines, BMP
Sean Brennan, MD, Cedar Hollow Pulmonary Assoc., (555) 014-2460	Fri 11/13/2026 2:00 PM	PFTs at 1:00 PM same day (arrive 12:45). Reassess inhaler regimen, ICS, O2 needs
Cedar Hollow Pulmonary Rehab, (555) 014-2475	Program will call to schedule within 2 wks	Pulmonary rehab intake (referral sent 10/24)
Tobacco Treatment Svc / state Quitline 1-800-QUIT-NOW	Call within 3 days	Quit support, NRT adjustment
Home O2 re-qualification (ambulatory oximetry)	Between 11/23 and 12/23/2026	Required to continue home O2 coverage; at PCP or pulm visit

RETURN PRECAUTIONS: CALL 911 OR RETURN TO ED FOR

Severe SOB at rest or unable to speak in full sentences; SpO2 < 88% on O2; lips or fingertips turning blue; new confusion or excessive sleepiness; chest pain; fast heartbeat > 120 at rest or fainting; coughing up blood, black/bloody stools or other serious bleeding (on apixaban); fever > 38.3 C (101 F).
Call PCP same day for: needing albuterol more than every 4 h; increased or discolored sputum; BG > 300 twice in a row or < 70; unable to urinate; swelling of legs.
Total time on discharge day: 42 min (> 30 min), incl. med rec, pt/family education and coordination of care.

Electronically signed by: Priya Raman, MD, Hospital Medicine | 10/24/2026 15:31
Dictated/prepared: 10/24/2026 14:10 Transcribed: N/A (direct entry)
cc: Maya Desai, MD (PCP), via fax (555) 014-3351; Sean Brennan, MD (Pulmonology); Patient portal

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